

**NORTHERN CALIFORNIA VA HEALTH CARE SYSTEM
HEALTH CARE FOR HOMELESS VETERANS (HCHV)
CONTRACTED EMERGENCY RESIDENTIAL SERVICES (CERS)
EMERGENCY HOUSING (Licensed Adult Care Facility/Board and Care)**

A. BACKGROUND

Ending homelessness among Veterans was established as a national priority by the Department of Veterans Affairs in November 2009 at the National Summit on Ending Veterans Homelessness. In June 2010, the U.S. Interagency Council on Homelessness (USICH) released *Opening Doors: Federal Strategic Plan to Prevent and End Homelessness*, which is fully aligned with this goal. Eliminating Veteran Homelessness remains one of three Agency Priority Goals for in VA's Strategic Plan.

The Health Care for Homeless Veterans (HCHV) program is an essential and critical part of VHA, providing a gateway to VA and community-based supportive services for eligible Veterans who are homeless. HCHV programs provide outreach services; care, treatment, and rehabilitative services, including case management services; and therapeutic transitional housing assistance under 38 U.S.C. 2032 in conjunction with Work Therapy under 38 U.S.C. 1718. The program uses Contracted Residential Services (CRS) in community locations to engage homeless Veterans who have been underserved. Many of these Veterans would benefit from a shelter program but will not avail themselves of these services without the encouragement of outreach workers.

B. PROGRAM DESCRIPTION

The HCHV CERS Program exists to provide a means of removing homeless Veterans from the street or habitation unfit for humans and placing them in community-based, residential environments with sufficient supportive services to meet their basic needs and ultimately, facilitate the improvement of their overall health status and housing situation.

This program will provide residential care including care for Acts of Daily Living (ADL) and Independent Acts of Daily Living (IADL) care for Veterans and must have an Adult and Senior Care Facility license (Residential Care Facility for the Elderly "RCFE") in the State of CA to provide ADL and IADL care. The program will also provide case management services to homeless Veterans in order to assist them to improve their income, health care, and access to long term permanent housing. The program residential facility must be located in Contra Costa, Solano, Yolo, Sacramento and West Placer Counties in California (See PWS Addendum 1 Zip Codes). The program will provide each Veteran room & board, private or shared bathroom and common area. This contract will be for between 1 and 5 beds, starting with 4 beds, for men and women. The program should have a minimum establishment of one-year in the required zip codes. Period of performance begins September 30th, 2026.

B.1. HCHV CERS Program characteristics:

B.1.1. Targets and prioritizes homeless Veterans who are eligible for VA health care, and are transitioning from chronic literal street homelessness, Veterans being discharged from institutions, and Veterans who recently became homeless and require safe and stable living arrangements while they seek permanent housing

B.1.2. Accepts referrals of homeless veterans that need Acts of Daily Living (ADLs) and Independent Acts of Daily Living (IADLs) assistance. Provides on-site care for ADLs and IADLs needs,

and holds a state license for that facility location to perform ADL and IADL assistance. Basic services will include the following:

- Private or shared rooms
- Meals and snacks (usually 3 meals/day)
- Housekeeping and laundry
- 24-hour supervision and staff availability
- Social and recreational activities
- Bathing
- Dressing
- Grooming
- Toileting
- Eating
- Transferring (e.g., getting in/out of bed)
- Storage and distribution of medications
- Assistance with self-administration
- Monitoring for changes in condition
- Routine cleaning
- Linen service
- General building upkeep
- 24-hour staffing
- Emergency response systems

B.1.3. Ensures a Veteran is not denied entry to HCHV CRS based solely upon length of current abstinence from alcohol or non-prescribed controlled substances, the number of previous treatment episodes, the time interval since the last program entry, the use of prescribed controlled substances. Ensures a Veteran is not denied entry to HCHV CRS based solely upon legal history, except in situations where the program excludes the admission of those with a sex offense history due to having children on-site or having a residential treatment license that prohibits admitting those with a sex-offense history.

B.1.4. Provides safe, secure, and wheelchair accessible housing as well as supportive services.

B.1.5. Lengths of Stay typically range from 30 to 90 days with the option to extend based on clinical need.

B.1.6. Provides case management for Veterans by setting up treatment plans that address barriers to housing. Goals of the treatment plan could include, for example, applying for more income in order to afford a local board and care, assisted living, or long term care facility, or developing permanent placement options in-house or with other licensed board and cares for when the Veteran exits the program and is no longer VA HCHV funded. Veterans are expected to meaningfully engage their case managers and available programming.

B.1.7. Emphasis is placed on achieving placement in permanent housing with supports, or long term residential programs, or other RCFEs that offer path to increased housing stabilization at the time of program exit, and reducing negative exits due to rule violations or other avoidable circumstances.

B.2. House Rules and Expectations

B.2.1. Rules focus on staff and resident safety:

- No buying or selling of alcohol or drugs in the facility
- No use of illicit drugs in the facility
- No sexual activity between residents
- No violence or threats of violence
- Honor nightly curfew

B.2.2. When possible, infractions are to be used to engage residents, not simply as grounds for service termination. Negative discharges will be monitored as a measure of program quality.

B.2.3. Veterans are expected to engage programming and maintain communication with case managers at all times around matters relating to admission, stay, and treatment

B.3. Admission Practices

B.3.1. Staff will assist residents with admission forms and eligibility determination with VA

B.3.2. Facility works to reduce barriers to admission:

- Accepts referrals throughout day if possible
- Flexibility with admission processes

B.4. Admission Criteria

B.4.1. Homeless Veteran in need of ADL and IADL assistance

B.4.2. Eligible for VA Health Care

B.5. Overview of Types of Services Available to Residents

B.5.1. Safe, Secure housing (includes shared laundry and restroom facilities)

B.5.2. Three daily nutritious meals

B.5.3. Case Management and Care Coordination Services

B.5.4. Transportation Services

B.5.5. Benefits Services

B.5.6. Enhancement of Independent Living Skills

B.5.7. Permanent/ Transitional housing search support

B.5.8. On-site care for ADLs and IADLs needs

C. PERFORMANCE WORK STATEMENT

C.1. OBJECTIVE

The Contractor shall provide emergency housing and supportive services for homeless Veterans in accordance with the HCHV CERS model requirements. Services are expected to consist of supportive, secure housing such as other RCFEs, or housing with support services for homeless Veterans. The

program places an expectation on the Veteran to engage in supportive case management services, and also on the contractor to provide expertise and guidance for improving housing stability to ensure a transition from homelessness to permanent, stable housing. Contractors must comply with all HCHV CERS requirements as identified below.

C.2. HCHV CERS PROVIDER QUALIFICATIONS & CAPABILITY REQUIREMENTS

- C.2.1.** Shall perform outreach or otherwise identifying and referring homeless Veterans on obtaining permanent housing, working on increasing income, and stabilizing medical and mental health;
- C.2.2.** Shall provide secure rooming and bathroom accommodations for males and Females. Males and Females should not share a bedroom or a bathroom at the same time. Common use of kitchen facilities and dining rooms is acceptable;
- C.2.3.** Shall provide services twenty-four (24) hrs. a day for Veteran placements lasting up to ninety (90) days per Veteran. Extensions beyond initial ninety (90) days should be authorized in writing by VA Liaison; extensions beyond six (6) months must be prior-approved by the Social Work Service Contract Coordinator/ COR;
- C.2.4.** Shall provide three daily nutritious meals and reasonable accommodation for special dietary needs;
- C.2.5.** Shall offer a means and assistance for clients to have their clothes washed or otherwise tend to laundry;
- C.2.6.** Shall provide secure, appropriate storage for both Veteran belongings and medication;
- C.2.7.** Shall provide quality case management and treatment services that utilize a Recovery Model approach that includes elements of motivational interviewing, harm reduction, and critical time intervention ;
- C.2.8.** Shall maintain a minimum of one staff member on site at all times to ensure appropriate response to matters involving Veteran safety;
- C.2.9.** Shall provide a working phone line and ensuring reasonably prompt communication with the VA is possible at all times;

C.3 DIRECT VETERAN CLINICAL SERVICES: 60% of time involves direct service to Veterans

- C.3.1. Occupancy:** The contractor will be responsible for ensuring that a minimally acceptable level of 80% occupancy (Preferred 90-100% occupancy) of HCHV CERS funded beds is maintained at all times through independent outreach efforts as well as by collaboration with VA.
- C.3.2 Care Planning:** Contractor will engage the veteran in a collaborative assessment of needs, including barriers to stable housing, and create an initial plan of care to address those concerns within 14 days of admission. This plan is to be updated as needed throughout the Veterans episode of care.
- C.3.3. Case Management:** Contractor will provide individual case management meetings at least

three (3) times per month (preferably once per week) that focus, at a minimum, on: housing search and stabilization, increasing income, any necessary coordination of ongoing participation in care provided by VA/community medical and/or mental health care providers, and ongoing discharge planning.

C.3.4. Exits to Housing: The contractor is expected to promote a focus on achieving stable housing or residential placement for all Veterans referred for care; exits to permanent housing will be monitored as an indicator of overall program quality utilizing data provided by the VHA Support Service Center's Homeless Service Scorecard – the target rate for exits to this type of housing will be 55% or higher (Accepted 50% or higher).

C.3.5. Negative Exits: The contractor is expected to facilitate Veteran completion of the HCHV CERS Program to the maximum extent possible, while still maintaining program integrity and safety; "Negative Exits," which shall be defined as discharges involving Veterans being asked to leave the program due to rule violation or otherwise leaving the program without consulting program staff in any way, shall be monitored on a continuous basis utilizing data provided by the VHA Support Service Center's Homeless Service Scorecard – the target rate for these types of exits will be 20% or less (Accepted 25% or less).

C.3.6. Environmental Checklist: The contractor will allow VA liaison to conduct unannounced site visits at a minimum of 4 times in a fiscal year, which includes, but is not limited to: inspecting the program facility, ensuring that there are no health code or safety violations, inspecting the medication storage area/room, and ensuring that Veterans are provided with 3 meals a day plus snacks, have adequate bedding, and storage for their belongings.

C.4. ADMINISTRATIVE SERVICES: 40% of time does not involve direct service to Veterans

C.4.1. Determination of Eligibility: The contractor is responsible for determining Veteran eligibility for placement into HCHV CERS Bed. A Veteran must be homeless and eligible for VA health care (Veterans Health Administration VHA eligible) in order to be considered for admission to any CERS Program. Failure to establish eligibility prior to admission may result in denial of payment for services provided to ineligible Veterans. In order to determine eligibility, contractor staff shall contact VA contract liaison during business hours in order to confirm eligibility. During after-business hours, weekends, and federal holidays contractor staff shall search the SQUARES database to determine possible eligibility and then confirm that eligibility the next business day with the VA liaison.

C.4.2. VA Information: Upon request Veterans' that are being admitted to the program that need any further verification of medical status and treatment will be provided by the VA. The agency must follow "Records Management Obligations," see Section K.

C.4.3. Individual Case Records: The contractor will maintain an individual case record for each referred Veteran. Case records must be maintained in security and confidence as required by the Confidentiality of Alcohol and Drug Abuse Patient Records (42 CFR part II) and the Confidentiality of Certain Medical Records (38 USC 7332), and in accordance with the Health Insurance Portability and Accountability Act (HIPAA: Pub. Law. 104–191). Records should contain at a minimum: Reason for referral; pertinent demographic information; copies of any medical prescriptions/orders issued by physicians; case management/treatment notes; any critical incident reports; and a final summary that include reason(s) for leaving, the Veteran's known future plans, and follow-up locator information, if available.

C.4.4. Homeless Management Information System (HMIS) Data Reporting: Veterans served through VA's HCHV CERS program must have client level data entered into the local Community Continuum of Care's (CoC) HMIS. Data entered must include, at a minimum, the Universal Data Elements from the 2010 HMIS Data Standards. This can be accessed through the local county government, sometime for a fee, sometimes for free. The VA will not reimburse this fee.

C.4.5. Daily Census Sign-In Sheet: The contractor is responsible for collecting Veteran participant signatures on a daily census sign-in sheet to verify attendance in the program. This information is to be provided to the VA Liaison on a regular daily or weekly basis in order to ensure effective monitoring of bed utilization.

C.4.6. Incident Reporting: The contractor shall notify the VA immediately when *any* adverse critical incident involving a Veteran admitted to the program occurs. Critical Incidents shall include:

- Falls
- Elderly/Dependent Adult Abuse or Neglect
- Sexual Assault
- Fire (Veteran Involved)
- Medical or Mental Health Emergency (911 Calls)
- Hospitalization
- Suicidal ideation or attempt
- Homicidal ideation
- Assault (of other residents or Staff)
- Death
- Infectious Control Concerns (Bed Bugs, TB, etc.)
- Observation/ Possession of Weapons

During normal business hours, the identified VA Liaison, HCHV Coordinator or COR should be contacted immediately by phone to report such an incident; if the incident occurs after hours, the local VA Facility Administrator on Duty (AOD) should be notified, also by telephone. A written report to the VA Liaison and/or COR should follow within 24 business hours. The report should list known precipitating factors or triggers. Contractor shall maintain a copy of all critical incident reports in the involved Veteran's individual case record.

C.4.7. Extension Requests: The contractor is responsible for ensuring that all requests for extension of services beyond the initially authorized service period of 90 days are reviewed and approved in writing by the VA Liaison (or Social Work Service Contract Coordinator/ COR for requests exceeding six (6) months).

C.4.8. Satisfaction Surveys: When requested, the contractor will be responsible for administering, collecting, and delivering to VA Liaison a survey of the Veterans experience in the program.

C.4.9. Discharge Reporting: The contractor is responsible for notifying the identified VA Liaison (or designee) within 24 business hrs. that a discharge has occurred, and for providing a written report of discharge that contains the specific data required by the VA for documentation of discharge in the Homeless Operations and Management Evaluation System (HOMES) within 48 business hrs.

C.4.10. Adult Care License: The contractor shall have and maintain a current CA state RCFE license for Adult and Senior Care Facility, so that ADL and IADL care can be provided.

D. DELIVERABLES

D.1. QUALITY CONTROL BUSINESS PLAN: The contractor shall submit a Quality Control Business Plan that supports the program objectives and associated tasks. The CO shall review and comment as necessary to ensure that contract goals are met.

D.2. STAFFING AND SERVICE PLAN: The contractor shall provide a detailed staffing and service plan. Plan should demonstrate that sufficient professional personnel are employed to carry out the policies, responsibilities, and services required under this contract. The Contractor must identify each person functioning as “Key Personnel” under this contract, and provide the VA with a description of the services to be provided by each person. The Contractor shall assign to this contract personnel that by education and/or training (and, when required, certification or licensure) are qualified to provide the services required by this PWS. Contractor may be required to supply a resume(s) summarizing relevant skills and experience of any/all key personnel upon request. Minimum Key Personnel requirements are as follows:

D.2.1. One administrative staff member, or designee of equivalent professional capability (with the authority to make decisions regarding the facility and residents, or with contact information for an offsite administrator in case of emergency), on duty on the premises or providing awake supervision of residents and staff 24 hours a day, 7 days a week.

D.2.2. Sufficient case management/counseling personnel to provide direct services to Veteran residents. Case managers should have some training and experience working with homeless individuals; experience working with individuals dealing with chronic medical, mental health and substance abuse problems is highly desirable. Staff working with Veterans should be able to assess, anticipate, and effectively refer Veterans experiencing crises for additional support as appropriate.

D.2.3. At least one staff or security member with CPR certification on site and available in an emergency during each shift, 24 hours per day. Staff should be aware of all DNR orders or lack thereof for each resident.

D.2.4. The contractor shall provide resumes for any proposed substitutions of key personnel, at least 15 days proposed date of substitution. substitution is to occur. The Contracting Officer shall notify the contractor within fifteen (15) calendar days after receipt of all required information if the VA is able to accept the proposed substitute key personnel. Temporary substitutions of key personnel shall be permitted in accordance with the contractor’s contingency plan. The contractor’s contingency plan to be utilized if personnel leave contractor’s employment or are unable to continue performance in accordance with the terms and conditions of the resulting contract should be submitted to CO as a part of proposal package. The CO is the ultimate authority on acceptable length for temporary substitution of key personnel.

D.2.5. The VA reserves the right to refuse or revoke acceptance of any key personnel if personal or professional conduct, or lack of required skills or experience jeopardizes patient care or interferes with the regular and ordinary operation of the facility and the HCHV CERS

Program.

- D.3. DOCUMENTATION OF SUPPORTIVE SERVICES:** The Contractor shall provide written documentation constituted by the individual Veteran case record that verifies the provision of all supportive services required under this contract for each Veteran participant.
- D.4. CARE PLAN:** The written plan of care shall be completed and entered into the individual Veteran case record no later than day 14 days after being admitted to the program.
- D.5. CRITICAL INCIDENT REPORTS:** Written critical incident reports must be submitted to the VA Liaison within 24 business hours (VA Liaison/AOD to be notified immediately or as soon as possible when incidents occur).
- D.6. EXTENSION REQUESTS:** Written requests for extension are due prior to the 90th day when it is anticipated the Veteran will require additional time beyond the initially authorized service period, and *prior* to the expiration of the initial and any and all subsequent future extension authorizations.
- D.7. DISCHARGE REPORTS:** The HOMES Exit form shall be completed and submitted to the VA Liaison within 48 business hours (VA Liaison to be notified within 24 hours of discharge).
- D.8. SATISFACTION SURVEYS:** When Veteran Resident surveys are being provided by VA to the program to give to Veterans, completed surveys are to be returned to the VA Liaison at the end of each month.
- D.9. DAILY CENSUS SIGN-IN SHEET:** The daily census sign-in sheet should be provided to the VA Liaison daily; weekend or holiday sign-in sheets are to be provided to the VA Liaison the on the next business day.
- D.10. INVOICES:** The monthly invoice is computed at the daily rate multiplied by the total number of beds occupied by Veterans at midnight each night of the given month. Invoices should first be submitted to the VA Liaison for approval and signature by the 5th of the month immediately following the billing period in question. Once approved, invoices are to be submitted through the Tungsten Network (Electronic Invoicing System) by the 10th of the month immediately following the billing period in question.; all electronic invoices submitted should be accompanied by invoice bearing VA Liaison signature for reference of certifying official. (For additional information, Reference: VAAR 852.273-72 Electronic Submission of Payments pg. 23. and FAR 52.232-33 Payments by Electronic Funds Transfer—System for Award Management pg.27.) .
- D.11. QUARTERLY PERFORMANCE REPORT:** The contractor shall provide the COR with a written report detailing program data and activities on a quarterly basis. The report should contain, at minimum, the following information:
- Total Number of Veterans Served
 - Occupancy rate
 - Percent of Veterans discharged to housing
 - Percent of negative discharges (i.e. discharges due to rule violation, failure to comply with program requirements, or unexpected discharges without prior consultation with staff).

- Other information the contractor feels pertinent, such as: quality improvement projects, changes in staffing or business practices, systems or resource concerns, etc.

DELIVERABLE TIME TABLE

Deliverables	Due Dates
D.1. QUALITY CONTROL BUSINESS PLAN	Due upon solicitation close date
D.2. STAFFING AND SERVICE PLAN	Due upon solicitation close date
D.3. DOCUMENTATION OF SUPPORTIVE SERVICES:	Completed Veteran case record due upon case close out; due to VA upon request only.
D.4. CARE PLAN	Due in Veteran case record by day 14; due to VA upon request
D.5. CRITICAL INCIDENT REPORTS	Due within 24 business hours of a critical adverse event involving a Veteran
D.6. EXTENSION REQUESTS	Due prior to the expiration of any authorized period of service
D.7. DISCHARGE REPORTS	Due within 48 business hours of Veteran discharge
D.8. SATISFACTION SURVEYS	Due at the end of each monthly service period
D.9. DAILY CENSUS SIGN-IN SHEET	Due daily
D.10. INVOICES	Due to VA Liaison by the 5 th of the month immediately following the billing period in question; due in Tungsten Network (electronic billing system) by the 10 th of the month immediately following the billing period in question.

Figure D.11.1

E. REFERRALS

E.1. The VA is capable in determining eligibility of Veterans prior to admission to contractor bed for services. The contractor is expected to work with identified VA Liaison or other

designees to confirm eligibility; it is understood that payment for Veterans admitted without an initial determination of eligibility may not be authorized if the Veteran is found to be ineligible.

E.1.2. A list of authorized VA ordering personnel, including primary identified VA Liaison, shall be made available to the contractor upon award of the contract. VA employees may be added or deleted from this list during the term of the contract at the discretion of VA. The contractor shall not deviate from the list of individuals authorized to approve admissions without an updated list.

E.1.3. Referral constitutes authorization of an initial service period of up to 90 days (unless otherwise specified).

F. ABSENCES AND CANCELLATION

F.1. The Contractor shall notify VAMC of any absences from the facility. Absences of the patient from the facility in excess of forty-eight (48) hours will not be reimbursable except those with the prior approval of the VAMC coordinator. Should a patient referred to a residential treatment facility absent himself/herself in an unauthorized manner, payment for services for that Veteran to the contract facility would be continued for a maximum period of two days provided there is an active outreach attempt on the part of the contractor facility staff to return the Veteran to the residential treatment program and a strong likelihood that the patient will return. Management of Negative Exits will be an element of quality assurance review of this program.

F.1.1. The VA will pay per diem up to a maximum of 48 consecutive hours for the unscheduled absence or 96 hours for scheduled absence of a Veteran under the following conditions:

1) **Scheduled Absences:** To receive payment, the absence must:

- a) Be pre-planned, consistent with and support the Veteran's individual service plan, (e.g., family reunification, short term medical, substance use disorder, (SUD), or psychiatric treatment).
- b) Have the reason be documented in the individual Veteran's case file, treatment record, or service plan.
- c) Not result in the bed being filled by the provider
- d) Not be for a break or vacation from treatment.
- e) Not be used for extended educational or employment circumstances.
- f) Not be used to create more than 4 consecutive days of absence.

2) **Unscheduled Absences:** To receive payment for an unscheduled absence:

- a) The provider must have evidence of active outreach to locate and reengage the Veteran and document the steps taken in the Veterans individual case file, treatment record, or service plan.
- b) The provider may not fill the bed.
- c) The Veteran must be discharged from the HCHV/CERS program if not located within 48 hours.

3) **Ineligible Veteran:** When a Veteran is admitted into an HCHV/CERS program and found to be ineligible for HCHV/CERS, VA will pay for a maximum of 4 days from the day of admission to allow the provider and HCHV/CERS liaison time to locate and arrange alternate placement.

F.2. The contractor may consider providing an authorized absence (or "pass") for purposes that are expected to further the recovery goals of a Veteran (e.g., job-related absences, family visits, housing searches, medical, etc.). All requests for passes must be documented in writing in the

Veterans individual case record. Authorized absences shall not exceed 48 hours in any given month for any single Veteran, unless otherwise approved by contractor *and* VA Liaison.

- F.3.** VA reserves the right to remove any or all Veterans from the facility at any time without additional cost, when it is determined to be in the best interest of the Veteran or VA.

G. CONDUCT

- G.1.** The contractor shall comply with the principles listed in 38 CFR 17.707(b) to provide housing and supportive services in a manner that is free from religious discrimination.
- G.2.** Local law enforcement and/or fire departments should be contacted for assistance and intervention as appropriate and indicated by any given circumstances.
- G.3.** The contractor shall also notify the VA Liaison, AOD, or COR immediately of any high risk situations involving veterans with suicidal and/or homicidal threats or ideation, episodes of physical or sexual violence, sexual activities, safety concerns, or activities involving illegal substances so that appropriate response may be coordinated.
- G.4.** In the event of a medical or psychiatric emergency, it is agreed that every effort will be made to facilitate Veteran access to local VA Medical Center for care. If a VA Medical Center is not available in the vicinity or is otherwise inconveniently located, the Contractor will advise the VA Liaison or AOD of the facility to which the Veteran has been admitted. The Contractor will also be expected to assist Veteran's requiring non-urgent services with accessing appropriate care from a VA or community facility, as appropriate.

H. COMPLAINTS

The identified VA Liaison and the Contracting Officer's Representative (COR) will monitor the services being provided in all HCHV CERS Facilities. The contractor is expected to cooperate with VA Staff and COR by providing information and answering questions in a timely manner when requested. Contractor shall refer complaints received directly from Veterans to the identified VA Liaison or COR within 48 hours of complaint. All complaints received by the VA Liaison or COR will be immediately forwarded to the contractor and shall be investigated promptly. After investigation and clarification of disposition, the contractor shall respond to the VA Liaison or COR within five (5) working days or less with proposed resolution or plan for corrective action. The CO shall be notified in instances where the proposed course of action or response does not appear sufficient to resolve any given complaint.

I. TRAVEL

The contractor is expected to assist Veterans with arranging local transportation to scheduled meetings and appointments. If contractor and VA staff both determine that public transportation is not available, adequate, or appropriate for any Veteran, the contractor will be expected to assist the Veteran with providing transportation to a Veteran directly.

J. FACILITIES

- J.1. General requirements:** It is the responsibility of the Contractor to properly maintain its facilities and the VA shall have no responsibility for paying or reimbursing the Contractor for such expenses. The contract facility must:

- J.1.1 Have a current occupancy permit issued by the local and state governments in the jurisdiction where the facility is located.
- J.1.2. Be in compliance with existing standards of State safety codes and local, and/or State health and sanitation codes.
- J.1.3. Meet the requirements of the Americans with Disabilities Act (ADA) (Public Law 100-336, 42 USC 12101-12213) pertaining to handicapped accessibility in effect on the date of contract award.
- J.1.4. Where applicable, be licensed under State or local authority.
- J.1.5. Where applicable, be accredited by the State.
- J.1.6. Be equipped with operational air conditioning /heating systems
- J.1.7. Be kept clean free of dirt, grime, mold, or other hazardous substances and damaged noticeably detract from the overall appearance.
- J.1.8. Be equipped with first aid equipment and an evacuation plan in case of emergency.
- J.1.9. Have windows and doors that can be opened and closed in accordance with manufacturer standards.
- J.1.10. Have an aggressive on-going plan to address bed bug infestation. This policy must be a part of your written response to this solicitation. On-going bed bug infestation will be grounds for immediate discharge of Veterans from the facility
- J.1.11. Have an emergency plan for potential situations when a facility becomes non-operational or unsafe and veterans need to be placed in an alternative location.

J.2. Fire Safety Requirements:

J.2.1. The building must meet the requirements of the applicable residential occupancy chapters of the current version of NFPA 101, National Fire Protection Association's Life Safety Code. Any equivalencies or variances must be approved by VANCHCS Director

J.2.2. Fire exit drills must be held at least quarterly. Residents must be instructed in evacuation procedures when the primary and/or secondary exits are blocked. A written fire plan for evacuation in the event of fire shall be developed and reviewed annually. The plan shall outline the duties, responsibilities and actions to be taken by the staff and residents in the event of a fire emergency. This plan shall be implemented during fire exit drills.
A written policy regarding tobacco smoking in the facility shall be established and enforced.

J.2.4. Portable fire extinguishers shall be installed at the facility. Use NFPA 10, Portable Fire Extinguishers, as guidance in selection and location requirements of extinguishers. Requirements for fire protection equipment and systems shall be in accordance with NFPA 101. All fire protection systems and equipment, such as the fire alarm system, smoke detectors, and portable extinguishers, shall be inspected, tested and maintained in accordance with the applicable NFPA fire codes and the results documented.

J.3 Inspection: Prior to the award of any contract and annually thereafter during any subsequent contracted performance periods, a multidisciplinary VA team consisting of a social worker, dietitian or nutrition and food service professional, nursing staff, VA Police, and a Safety and Occupational Health Specialist, as well as any other subject matter experts determined necessary by the medical center director, COR, HCHV Coordinator, or VA Liaison, shall conduct a survey of the contractor's facilities to be used to provide Veterans food, shelter, and clinical services to assure the facility provides acceptable level quality care in a safe environment. Additional inspections may also be carried out, announced or unannounced at any other time as deemed necessary by VA, with a minimum of 4 times a year. (See PWS Addendum 2 Copy of Blank Inspection Form)

The contractor will be advised of the findings of the inspection team. If deficiencies are noted during any inspection, the contractor will be given a reasonable amount of time (typically 30 days) to take corrective action and to notify the Contracting Officer that the corrections have been made. A contract will not be awarded until noted deficiencies have been eliminated. Failure by the Contractor to take corrective action within the reasonable time provided will be reported to the VA Contracting Officer. If corrections are not made to the satisfaction of the VA, the Contracting Officer will be notified, and shall be the final arbiter on the necessary resulting consequences and action.

The inspection of the Contractor facilities will include inspection for conformity to the current Life Safety Code as described in paragraph 5, and will also include the following:

J.3.1. General observation of residents to determine if they maintain an acceptable level of personal hygiene and grooming.

J.3.2. Assessment of whether the facility meets applicable fire, safety and sanitation standards.

J.3.3. Determining whether the facility is in attractive surroundings conducive to social interaction and the fullest development of the resident's rehabilitative potential.

J.3.4. Observation of facility operations to see if appropriate organized activity programs are available during waking hours (including evenings) and degree to which a high level of activity is observed in the facility, such as individual professional counseling, physical activities, assistance with health and personal hygiene.

J.3.5. Seeking evidence of facility-community interaction, demonstrated by the nature of scheduled activities or by information about resident flow out of the facility, e.g., community activities, volunteers, local consumer services, etc. This should include not less than one Veteran or formally homeless Veteran on the board of directors or the equivalent policy making entity. It should also include attempts to involve homeless Veterans through employment, volunteer services, or otherwise in construction, rehabilitation, maintaining, and operation of the program.

J.3.6. Observation of staff behavior and interaction with residents to determine if they convey an attitude of genuine concern and caring.

J.3.7. Inspecting the types of meals and other nutrition provided to residents to see if appetizing, nutritionally adequate meals are provided in a setting, which encourages social interaction and if

nutritious snacks between meals and bedtime are available for those requiring or desiring additional food, when it is not medically contraindicated.

J.3.8. Making a spot check of Veterans' records to ensure accuracy with respect to Veterans' length of stay and services provided to the Veterans.

All Department of Veterans Affairs inspection findings for residential facilities furnishing treatment and rehabilitative services to eligible Veterans shall, to the extent necessary, be made available to all government agencies charged with the responsibility of licensing or otherwise regulating or inspecting such institutions.

K. CERTIFICATION & ACCREDITATION REQUIREMENTS

Acquisition a service that involves the storage, generating, transmitting, or exchanging of VAI does not require system interconnection.

RECORDS MANAGEMENT OBLIGATIONS

A. Applicability

This clause applies to all Contractors whose employees create, work with, or otherwise handle Federal records, as defined in Section B, regardless of the medium in which the record exists.

B. Definitions

"Federal record" as defined in 44 U.S.C. § 3301, includes all recorded information, regardless of form or characteristics, made or received by a Federal agency under Federal law or in connection with the transaction of public business and preserved or appropriate for preservation by that agency or its legitimate successor as evidence of the organization, functions, policies, decisions, procedures, operations, or other activities of the United States Government or because of the informational value of data in them.

The term Federal record:

1. includes VHA records.
2. does not include personal materials.
3. applies to records created, received, or maintained by Contractors pursuant to their VHA contract.
4. may include deliverables and documentation associated with deliverables.

C. Requirements

1. Contractor shall comply with all applicable records management laws and regulations, as well as National Archives and Records Administration (NARA) records policies, including but not limited to the Federal Records Act (44 U.S.C. chs. 21, 29, 31, 33), NARA regulations at 36 CFR Chapter XII Subchapter B, and those policies associated with the safeguarding of records covered

by the Privacy Act of 1974 (5 U.S.C. 552a). These policies include the preservation of all records, regardless of form or characteristics, mode of transmission, or state of completion.

2. In accordance with 36 CFR 1222.32, all data created for Government use and delivered to, or falling under the legal control of, the Government are Federal records subject to the provisions of 44 U.S.C. chapters 21, 29, 31, and 33, the Freedom of Information Act (FOIA) (5 U.S.C. 552), as amended, and the Privacy Act of 1974 (5 U.S.C. 552a), as amended and must be managed and scheduled for disposition only as permitted by statute or regulation.

3. In accordance with 36 CFR 1222.32, the Contractor shall maintain all records created for Government use or created in the course of performing the contract and/or delivered to, or under the legal control of the Government and must be managed in accordance with Federal law. Electronic records and associated metadata must be accompanied by sufficient technical documentation to permit understanding and use of the records and data.

4. VHA and its contractors are responsible for preventing the alienation or unauthorized destruction of records, including all forms of mutilation. Records may not be removed from the legal custody of VHA or destroyed except in accordance with the provisions of the agency records schedules and with the written concurrence of the Head of the Contracting Activity. Willful and unlawful destruction, damage, or alienation of Federal records is subject to the fines and penalties imposed by 18 U.S.C. 2701. In the event of any unlawful or accidental removal, defacing, alteration, or destruction of records, the Contractor must report to VHA. The agency must report promptly to NARA in accordance with 36 CFR 1230.

5. The Contractor shall immediately notify the appropriate Contracting Officer upon discovery of any inadvertent or unauthorized disclosures of information, data, documentary materials, records,

or equipment. Disclosure of non-public information is limited to authorized personnel with a need-to-know as described in the [contract vehicle]. The Contractor shall ensure that the appropriate personnel, administrative, technical, and physical safeguards are established to ensure the security and confidentiality of this information, data, documentary material, records, and/or equipment is properly protected. The Contractor shall not remove material from Government facilities or systems, or facilities or systems operated or maintained on the Government's behalf, without the express written permission of the Head of the Contracting Activity. When information, data, documentary material, records, and/or equipment is no longer required, it shall be returned to VHA control or the Contractor must hold it until otherwise directed. Items returned to the Government shall be hand-carried, mailed, emailed, or securely electronically transmitted to the Contracting Officer or address prescribed in the [contract vehicle]. Destruction of records is EXPRESSLY PROHIBITED unless in accordance with Paragraph (4).

6. The Contractor is required to obtain the Contracting Officer's approval prior to engaging in any contractual relationship (sub-contractor) in support of this contract requiring the disclosure of information, documentary material, and/or records generated under, or relating to, contracts. The Contractor (and any sub-contractor) is required to abide by Government and VHA guidance for protecting sensitive, proprietary information, classified, and controlled unclassified information.

7. The Contractor shall only use Government IT equipment for purposes specifically tied to or authorized by the contract and in accordance with VHA policy.

8. The Contractor shall not create or maintain any records containing any non-public VHA information that is not specifically tied to or authorized by the contract.

9. The Contractor shall not retain, use, sell, or disseminate copies of any deliverable that contains information covered by the Privacy Act of 1974 or that which is generally protected from public disclosure by an exemption to the Freedom of Information Act.

10. The VHA owns the rights to all data and records produced as part of this contract. All deliverables under the contract are the property of the U.S. Government for which VHA shall have unlimited rights to use, dispose of, or disclose such data contained therein as it determines to be in the public interest. Any Contractor rights in the data or deliverables must be identified as required by FAR 52.227-11 through FAR 52.227-20.

11. Training. All Contractor employees assigned to this contract who create, work with or otherwise handle records are required to take VHA-provided records management training. The Contractor is responsible for confirming training has been completed according to agency policies, including initial training and any annual or refresher training.

[Note: To the extent an agency requires contractors to complete records management training, the agency must provide the training to the contractor.]

D. Flow down of requirements to subcontractors

1. The Contractor shall incorporate the substance of this clause, its terms, and requirements including this paragraph, in all subcontracts under this [contract vehicle], and require written subcontractor acknowledgment of same.

2. Violation by a subcontractor of any provision set forth in this clause will be attributed to the Contractor.

L. CORONAVIRUS 2 (SARS-CoV-2) and other infectious diseases in HCHV CRS Programs

L.1. HCHV CRS programs shall have protocols for action to prevent and protect against COVID-19 and other infectious diseases. HCHV CRS programs shall coordinate with the VA to mitigate outbreaks in congregate settings. HCHV CRS programs must follow local guidelines and recommendations and shall work closely with local and state health departments. VHA strongly encourages all Veterans and staff to be vaccinated, stay up to date with vaccinations, including recommended boosters, and utilize other forms of prevention and protection.

L.2. HCHV CRS programs shall review the CDC list of Symptoms of COVID-19 and other infectious diseases and specifically guidance on when to seek emergency medical attention.

L.3. Sites are required to have processes in place to ensure available housing options for isolation and quarantine to mitigate any gaps in housing and related resources by utilizing the Options for Social Isolation Under the COVID-19 National Emergency: Guidance for HCHV CRS during the PHE:

Isolation Option 1: CRS Provider has space at existing location

- Provider is proposing to use additional space within their current site (second floor unoccupied space or staff office that has now been turned into isolation area).

- The site has been inspected; HCHV Liaison should ensure the contract Contracting Officer's Representative (COR) and CO are aware and the liaison and provider should maintain local records.

Isolation Option 2: CRS Provider does not have space at existing location

- Provider proposes a new site that is not currently a site include in their contract, but that their agency has access to.
- HCHV Liaison and/or COR request that the CO completes an expeditious contract modification with the knowledge that the provider is willing to do so.
- HCHV Program Office guidance regarding modified inspection process may be used to facilitate rapid movement of Veterans with the agreement of the CO.
 - o Inspections may be conducted virtually, as applicable and acceptable to the CO, using video. The inspector and VAMC Director must still sign the inspection package.
 - o If the site was previously inspected and approved within the last 12 months for GPD, this inspection will be accepted for temporary placement of Veterans in HCHV CRS, with approval of the CO.

Isolation Option 3: Use of another HCHV CRS facility

- CRS Provider does not have space, but another existing HCHV CRS Provider does and is willing to take the Veteran.
- The Veteran can be transferred to that location with discharge from the current HCHV CRS program and admission to the new HCHV CRS program.
- No inspection needed because site was already inspected as an existing HCHV CRS Provider.

Isolation Option 4: Use of local hotel/motel room

- Veterans may be placed in a hotel/motel room for COVID-19-related reasons, including facilitating isolation, quarantine, social distancing efforts, and/or controlling an outbreak. Time-limited use of hotels/motels may occur during the pandemic to facilitate rapid movement of Veterans into safe locations.
- The HCHV Liaison and/or the COR request that the Contracting Officer complete an expeditious review and required contract modification with the knowledge that the provider has a plan to utilize this quarantine/isolation option.
 - o In order to use hotel/motel space for this purpose, the provider must have a plan to check on the Veteran daily (remote contact allowed) for wellness checks and supportive services and to facilitate meals.
- Veterans in hotels/motels are required to be counted in the monthly census and providers may not exceed their authorized number of beds. Providers may bill at their authorized per diem rate for the bed days of care provided.